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Safety of Elderly Patients in Hospitals

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The world's population is ageing fast. In 2020, 1 billion people in the world were aged 60 years or over. That figure will rise to 1.4 billion by 2030, representing one in six people globally. By 2050, the number of people aged 60 years and over will have doubled to reach 2.1 billion. The number of persons aged 80 years or older is expected to triple between 2020 and 2050 to reach 426 million (4)

According to NITI Aayog report, in India, senior citizens, i.e. people aged 60 years and above, currently comprise a little over 10% of the population, translating to about 104 million. Life expectancy in our country is 67.3 years in 2021. The United Nations Population Fund (UNFPA) projects that this population, which will make up 158 million people by 2025, is the one that is ageing at the fastest rate. By 2050, the elderly population is projected to rise to 319 million (19.5% of the total population).

Further, the total dependency ratio (ratio of people who are economically dependent on others to the no of people able to provide support), which stood at 56.92 in 2020, is projected to decrease steadily till the 2025 due to the rise in the percentage of the working-age population; but it is expected to rise again to touch 61.22 by 2050.

Government concern about the health of elderly, so in a significant move, the Union Cabinet approved a major expansion of the Ayushman Bharat Pradhan Mantri Jan Arogya Yojana (AB PM-JAY) on September 11, 2024. Under this decision, all senior citizens aged 70 and above will receive health coverage, regardless of their income, providing them with free health insurance coverage of up to Rs 5 lakh per family.

Key Areas of Concern

a. OPD Visit:

1. The hospital may provide dedicated registration counter for elderly patients.
2. Lobby should be well lit and light should adequate

3. "May I help you" counter who actively listen to their problem and try to solve on priority. This signage must be in local language too.
4. Many elderly can't walk long in the corridor- so availability of adequate wheel chairs is necessary.

5. Easily accessible lift, drinking area and toilets.

6. Pharmacist at medicine counter to explain proper intake of medicine.

7. Clear instructions when to visit next and red flag signs when they can visit hospital anytime. Many times, physician call patients very frequently for observing blood pressure and sugars. We should avoid this and ask them to make a chart at home and visit we any value deviate much from normal.

8. There should be a separate facility of getting investigations for elderly, physically challenged

persons and report collection too.

9. Proper signage must be there at appropriate place, so that it's maximum visible and appreciated by the people.

10. Vaccination: Promote immunizations for preventable diseases, such as influenza and pneumonia

11. The hospital shall run dedicated geriatric clinics with trained/ qualified health care staff.

b. IPD care

Geriatric persons are not just more aged adults. They are fragile and all organs are at their age, so they compromise early in comparison to young persons. There are few areas where we should more emphasise when deal with elderly:

- Communication Barriers: Hearing, vision, or cognitive impairments can hinder effective communication. This many times lead to over diagnosis of altered sensorium. Delirium in elderly is important to recognise as it can signify severe sepsis, electrolyte imbalance, and CVA etc.
- Falls: Elderly patients are at high risk of falls, leading to fractures and other complications. Elderly and frail are at risk of fracture which is common in hospital setting. Restrain is another risk factor for soft tissue and fractures in elderly in hospital. Training and education for use of toilets to be done.
- Medication Errors: Polypharmacy increases the chances of drug interactions and incorrect dosages.
- Infections: Prolonged hospital stays and invasive procedures elevate the risk of healthcare-associated infections (HAIs). Elderly are at risk of developing bed sores if care is not taken. In ward patient attendant can be a part of patient care along with nursing.
- Emotional Well-being: Anxiety, depression, and feelings of isolation are common among elderly patients.
- Restrain: The Restraint Policy NABH serves as a guideline for healthcare institutions to manage patients who may pose a risk to themselves or others. The policy covers the appropriate use of physical, mechanical, or chemical restraints while focusing on minimizing harm and respecting the patient's rights.

Restraint is only used as a last resort after other de-escalation techniques have failed. NABH emphasizes that patient safety and dignity should be the top priority when implementing any restraint. The policy ensures that healthcare providers have a clear process to follow, ensuring compliance with legal and ethical standards.

- Use of ID bands: All elderly patients in the hospital shall have to be considered as vulnerable patients and appropriate preventive measures as per need may be adopted by the hospital. Patient and family education about vulnerability and preventive aspects to be done by the nursing officers. Side rails of bed to be raised. Vulnerable patient ID band to be used.

- Follow up Tele-Consultation: Tele-consultation services may be provided for follow up of geriatric patients wherever applicable in order to prevent frequent visits to hospitals (both for OPD and IPD patients)

Policies to Uplift care for Elderly in Hospital

1. Communication Training

- Staff Education: Conduct workshops on geriatric care and effective communication techniques.
- Sign Language Interpreter: Arrange for a sign language interpreter if the patient uses sign language as their primary communication method.
- Patient-Centred Communication: Use simple language, visual aids, and ensure interpreters are available if needed.
- Simple Language: Use short sentences, avoid medical jargon, and repeat information as needed.
- Nonverbal Cues: Use gestures, facial expressions, and body language to reinforce verbal communication.

2. Environmental Adjustments:

- Anti-skid tiles, call bell system and hand rails in bathrooms and toilets
- Adequate Lighting: Ensure rooms and examination areas are well-lit without glare.
- Large Print Materials: Provide written materials in large fonts or Braille, depending on the patient's needs.
- Contrast and Colour Coding: Use high-contrast colours for signage, instructions, or equipment to make them easier to identify.
- Family Involvement: Engage family members in care plans to bridge communication gaps.

3. Involve Caregivers

- Family as Partners: Engage family members or caregivers to support understanding and decision-making.
- Behavioural Cues: Observe input from family about the patient's usual behavior or communication preferences.

4. Technology-Based Solutions

- Video Remote Interpreting (VRI): Use video interpreting services for patients with hearing loss.
- Text-to-Speech Apps: Provide tools that convert text to speech for patients with vision impairments.
- Cognitive Support Apps: Use apps that send reminders for medication or appointments.

5. Staff Training and Sensitization

- **Empathy Training:** Educate staff on the emotional and physical challenges faced by elderly patients with impairments.
- **Specialized Training:** Conduct workshops on communicating effectively with patients with sensory or cognitive impairments.
- **Cultural Competency:** Understand and respect the cultural and linguistic preferences of elderly patients.
- **Social Workers:** Engage social workers to help coordinate additional resources or caregiver support.

6. Enhancing Physical Safety

- **Fall Prevention:** Install non-slip flooring, handrails, and adequate lighting. Conduct fall-risk assessments for all elderly patients upon admission.
- **Assistive Devices:** Escorts, provide wheelchairs, walkers, and other aids as needed.
- **Regular Monitoring:** Increase the frequency of patient rounds, especially at night.
- **Surveillance:** Use CCTV cameras to monitor potential fall-prone areas and respond proactively.
- **Feedback Systems:** Collect patient feedback to identify safety concerns and implement improvements.

7. Medication Safety

- **Comprehensive Reviews:** Perform regular medication reconciliation to avoid polypharmacy risks.
- **Clear Labelling:** Ensure medications are clearly labelled with instructions, including pictorial aids if necessary.
- **Education:** Train staff to recognize signs of adverse drug reactions.
- **Evidence based treatment guidelines:** Hospital should adopt standard treatment guidelines from recognised bodies in order to prevent poly pharmacy and maintaining high standard of care.

8. Infection Control

- **Hygiene Practices:** Enforce strict hand hygiene protocols among staff, patients, and visitors.
- **Screening and Isolation:** Identify and isolate infections promptly to prevent cross-contamination.
- **Vaccination:** Promote immunizations for preventable diseases, such as influenza and pneumonia.

9. Promoting Emotional Well-being

- **Therapeutic Activities:** Offer recreational therapy sessions, including Yoga, music, art, and group activities.
- **Counselling Services:** Provide access to mental health professionals for emotional support.
- **Visitation Policies:** Implement flexible visitation hours to reduce feelings of isolation.

Geriatric and Palliative Medicine (GAP)-ED partnership

The National Programme for the Health Care for the Elderly (NPHCE) is an articulation of the International and national commitments of the Government as envisaged under the UN Convention on the Rights of Persons with Disabilities (UNCRPD), National Policy on Older Persons (NPOP) adopted by the Government of India in 1999 & Section 20 of “The Maintenance and Welfare of Parents and Senior Citizens Act, 2007” dealing with provisions for medical care of Senior Citizen. The primary aim

of this is to coordinate care for older adults who are likely to be discharged home.

Ensuring the safety of elderly patients in hospitals is a shared responsibility requiring concerted efforts from all stakeholders. Implementing the proposed strategies can significantly improve the quality of care and outcomes for this vulnerable population.

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